Cleveland Clinic

INTERNATIONAL & EMERGING MARKETS | ADVISORY OFFERING PROFILE

Financial Performance Improvement & Profitability

An advisory offering that shows leaders where margin is being lost, sizes the opportunities to recover it, and builds the cost foundations to see profitability by service line.

CATEGORY
Finance Advisory: Measure and Optimize Stages (understanding performance and improving margin)

TYPICAL DURATION
4 to 8 weeks; 12 to 24 weeks with service line profitability

DELIVERY MODEL
On-site, Hybrid, Workshop-based

01 CLIENT NEED

Boards and executives facing margin pressure from rising labor, supply and operating costs need an objective view of where performance is falling short and why. Many organizations lack reliable benchmarks, consistent productivity measures or the cost accounting needed to know which procedures, patients and service lines generate or consume margin once direct and indirect costs are fully considered.

This offering spans the Measure and Optimize stages of the Cleveland Clinic Healthcare Financial Advisory Lifecycle, which follows an organization's financial decisions through five stages: Plan, Measure, Optimize, Manage and Transform. These stages cover understanding current financial performance and improving margin toward best-in-class, and answer the client questions "How are we performing?" and "How do we become best-in-class?" It is typically requested by organizations facing margin pressure, from growing providers with limited performance data to mature health systems pursuing sustained margin improvement.

02 CLEVELAND CLINIC VALUE PROPOSITION

This offering is delivered by Cleveland Clinic's Finance Advisory team. Cleveland Clinic is an operating health system with 23 hospitals, 300 outpatient locations and about 83,000 caregivers, and has grown from Northeast Ohio into Florida, Las Vegas, Toronto, London and Abu Dhabi (figures as of December 31, 2025).

Clients get an operator's view of margin. Cleveland Clinic balances financial sustainability with clinical quality across a large system, so opportunities are tested for what is achievable without compromising care, and the cost accounting foundations are those a working health system uses to manage service line performance.

Cost rules, models and the improvement roadmap are built with the client's team and handed over with documentation, so improvements can be sustained after the engagement.

03 KEY DELIVERABLES

Every engagement includes the first five deliverables below, which make up the diagnostic. The cost accounting framework and service line profitability model are added when that extended scope is selected.

- **Current-state assessment:** written report on financial performance, covering key performance indicators (KPIs), the operational processes behind them and the main drivers of results
- **Benchmarking analysis:** comparison of financial and productivity measures against agreed peer and internal standards, with commentary on each material variance
- **Cost opportunity analysis:** quantified savings estimates, ranked by value and ease of implementation
- **Revenue opportunity analysis:** quantified opportunities in payer mix, service mix, pricing and charge capture
- **Improvement roadmap:** prioritized action plan with an owner, timing and estimated financial impact for each action, presented at an executive readout
- **Cost accounting framework (extended scope):** allocation rules for direct, indirect, fixed and variable costs, with supporting governance and documentation
- **Service line profitability model (extended scope):** margin by service line, with the cost and revenue detail behind each

04 DELIVERY MODEL

On-site / Hybrid / Workshop-based

Delivered by Cleveland Clinic's Finance Advisory team, working alongside the client's finance lead. The engagement begins with a structured data request, a review of financial results, cost data and operating processes, and interviews with finance, operational and clinical leaders. Analysis continues in weekly virtual working sessions, with on-site discovery added when cost accounting or service line profitability is in scope. Findings are validated with clinical and finance leaders before the final executive readout.

05 TYPICAL DURATION

4 to 8 weeks; 12 to 24 weeks with service line profitability

- **Discovery (1 to 2 weeks):** Data request, review of financial results and processes, and interviews with finance, operational and clinical leaders
- **Analysis and benchmarking (2 to 4 weeks):** Benchmarking and sizing of cost and revenue opportunities, reviewed in weekly working sessions
- **Validation and readout (1 to 2 weeks):** Opportunities validated with clinical and finance leaders, roadmap finalized and presented
- **Extended scope build (8 to 16 weeks):** Cost accounting rules designed and applied, service line profitability model built and tested, then handed over with training

The diagnostic runs 4 to 8 weeks, depending mainly on the number of hospitals or entities in scope and the quality of available cost and productivity data. The extended scope adds 8 to 16 weeks (12 to 24 weeks in total), driven mainly by the number of service lines and how complete the client's cost data is.

06 RESOURCE REQUIREMENTS

CLEVELAND CLINIC TEAM

- Director / Executive Sponsor: 5 to 10% (larger engagements)
- Assistant Director / Engagement Lead: 10 to 25%
- Manager / Workstream Lead: 20 to 40% (mid-sized and larger engagements)
- Senior Analyst: 30 to 60%
- Analyst: 20 to 50%
- Admin / Project Support: up to 10%

CLIENT TEAM

- Executive sponsor with decision authority, for scoping, key reviews and the final readout
- Finance lead as day-to-day point of contact, attending the weekly working sessions
- Operational and clinical leaders for interviews and to validate findings
- Cost accounting or decision support analyst when service line profitability is in scope
- IT or data team support for system extracts

Percentages are the share of each team member's working time during the engagement, so they do not add up to 100%. Roles marked for larger engagements join only when the scope calls for them.

ENGAGEMENT ASSUMPTIONS

- The client provides timely access to the financial, operational, market, volume, stakeholder, reporting and staffing information in the data request
- Scope and timing may change if data is late or incomplete
- Peer groups and benchmark sources are agreed at scoping

- Stakeholders are available for interviews and working sessions, and draft deliverables are reviewed within an agreed turnaround
- Recommendations are advisory; decisions and implementation remain with the client's leadership

07 EXPECTED OUTCOMES

- Leadership sees where margin is being lost and which drivers explain the gap to peers
- Cost and revenue opportunities are quantified and ranked, with an owner for each of the highest-value actions
- Improvement effort goes to changes that are achievable without compromising quality of care
- With the extended scope, leaders see true margin by service line and use it in investment decisions
- The client's team can sustain the work using the cost rules, governance and roadmap handed over